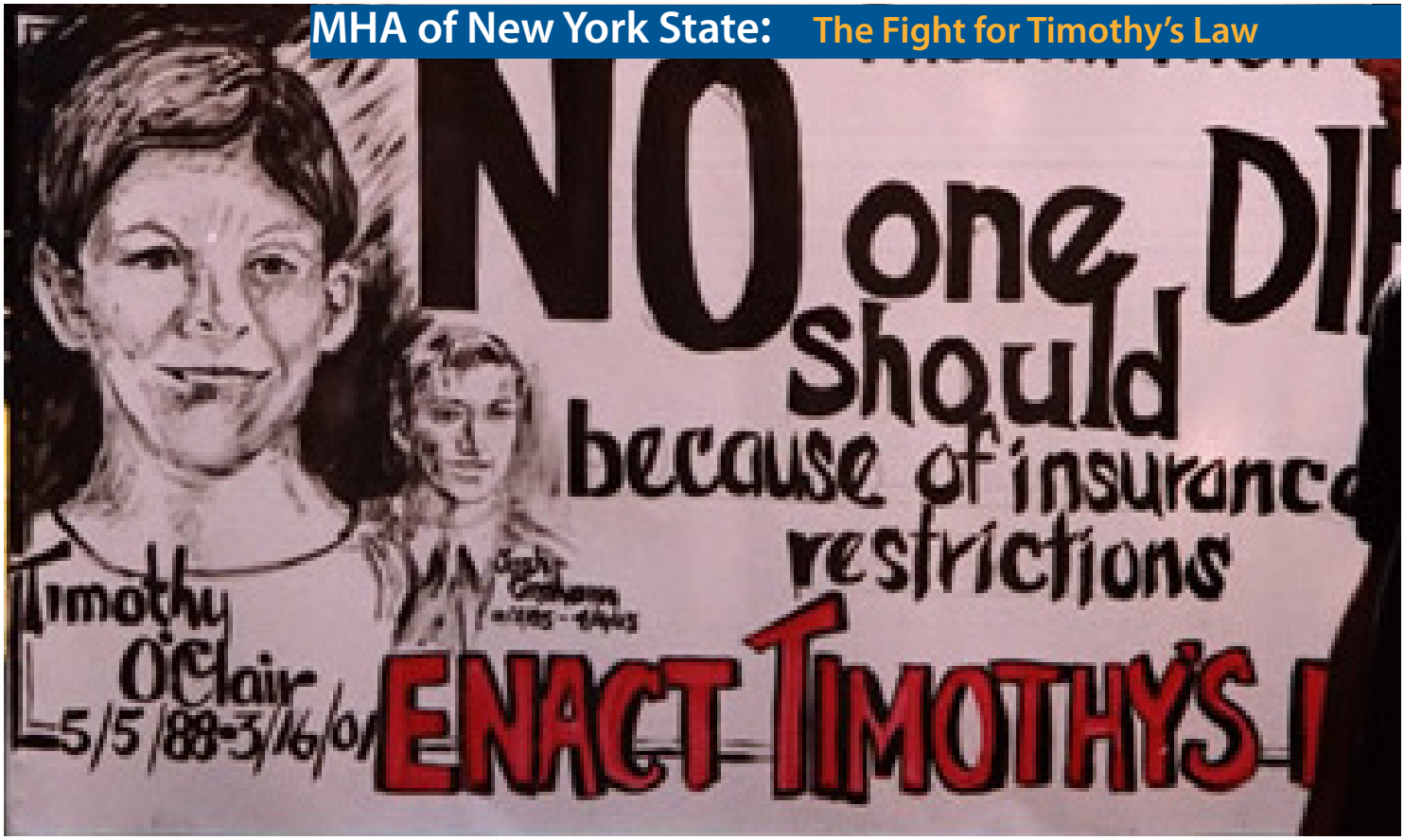
MHA of New York State: The Fight for Timothy's Law



The concept of mental health parity has been around for a long time. While with recent victories for mental health coverage through the Affordable Care Act and the finalization of parity regulations have made mental health parity a standard across the Country (as well as early victories through the Wellstone-Domenici Bill), there was a time in the not too distant past when mental health parity was still an alien concept to so many people. New York State was no exception as many advocates had fought for over two decades to make parity a reality.

Through the two decade fight for parity, the Mental Health Association in New York State (MHANYS) has always played a leading role in advocating for these changes through the various coalitions that were created over the years. However mental health parity in New York did not become a reality until it became a very personal journey to a father in Schenectady, New York—Tom O'Clair.

Courageously Tom shared the story of the suicide completion of his son Timothy at age 12. The story was heartbreaking--made even more so by the fact that the O'Clairs had to give up custody of their son to foster care so that Medicaid could pay for the cost of mental health treatment. Relinquishing custody of your child so that Medicaid could pay for mental health services is not a decision that any parent should have to make. If there was mental health parity in New York, the O'Clairs would have been able to afford the care that might have well-kept Timothy alive. This story of Timothy resonated across New York State. Many other families and individuals shared their own heart wrenching stories of losing their loved ones because there was no equality in treatment coverage between physical and mental health care.
In the minds of the Timothy's Law Coalition, this was a pure matter of discrimination. Why should someone have co-pays that continued to slide upwards for mental health treatment when they were not subject to those same kinds of fluxuations in insurance costs for a physical health condition. The premise was simple—there should not be disparate coverage between mental and physical illness.

Advocates had both a sound logic and a true leader in Tom O'Clair. What was needed was to build our base. MHANYS was very involved in the building of the coalition and its CEO was honored to serve as the co-chair of a coalition that not only included MHAs, but the Psychiatric Association, the New York State Association of Psychiatric Rehab Services, Coalition for the Homeless (who also brought in their incredibly savvy and respected lobbyist), NASW, Families Together in New York State, NAMI, The New York State Rehabilitation Association, Association of Alcoholism and Substance Abuse Providers, American Foundation for Suicide Prevention and several other organizations.

We also needed strong legislative support. By a lucky happenstance, Tom O 'Clair's local Assembly member was Paul Tonko who became a true champion. Assemblyman

Tonko (now in Congress) was a tireless advocate for support of this legislation. He and leaders like Patrick Kennedy continue to keep this issue alive nationally.

In the past the State Senate had opposed Timothy's Law using the argument that costs would skyrocket if you included mental health parity as part of insurance coverage. This was largely refuted by several studies, several of which showed mental health parity increased insurance cost by a little more than a dollar a month. Through the media efforts of Tom O'Clair and Timothy's Law Coalition members as well as grass roots campaign including dozens of rallies across New York State, many of them led by MHA affiliates in their region, there became a groundswell of support that legislators could not ignore. On the last day of legislation session in Albany, one Senator who was long opposed to Timothy's Law came over to our coalition members and said he received over 2500 phone calls the previous day from his constituents and he was bending to their will by supporting the law. This was as microcosm of the whole Senate and in one of their last votes before adjourning for the session, they passed Timothy's Law.

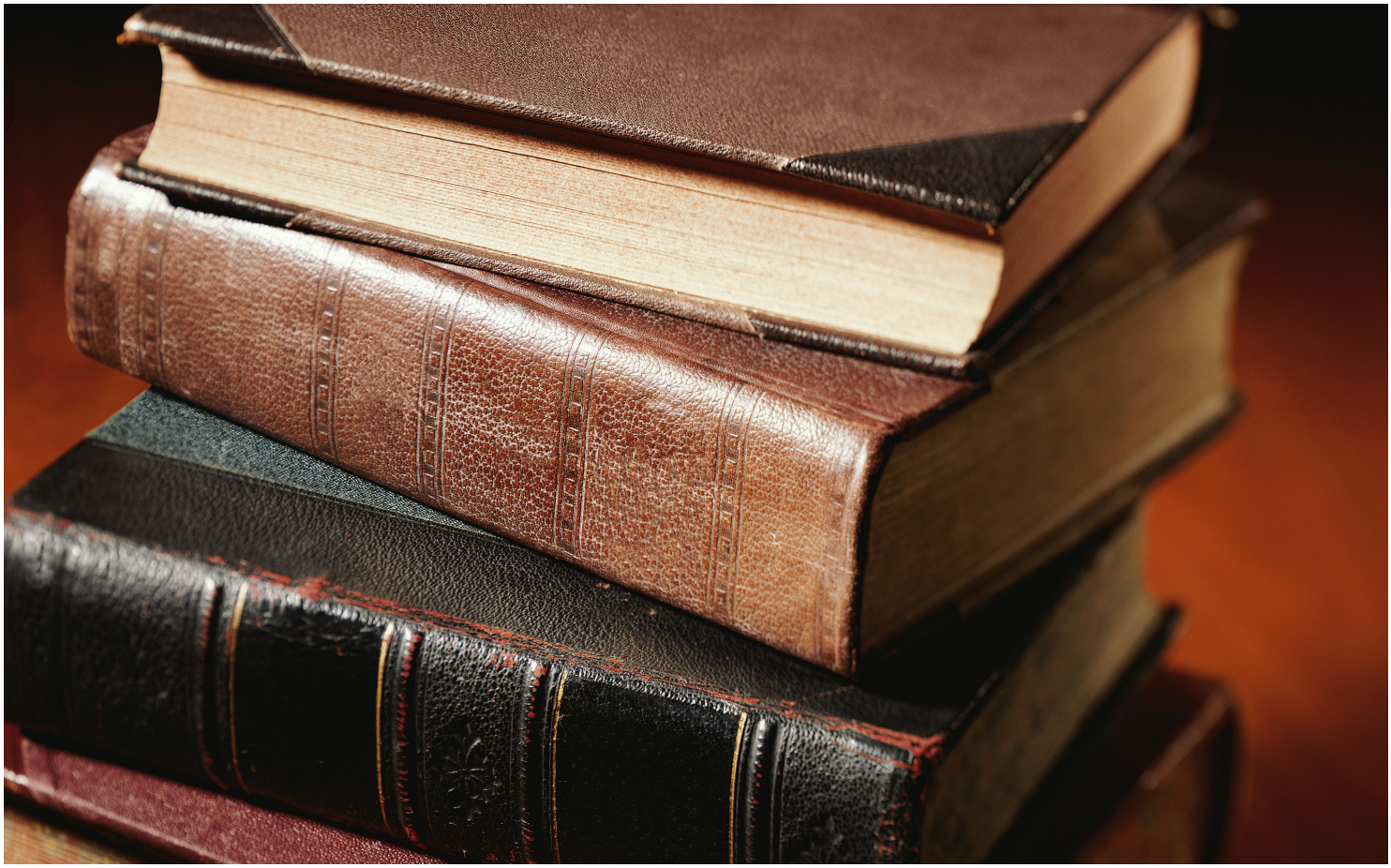
After both the Assembly and Senate passed the laws, it still had to be signed by Governor George Pataki who had never publicly supported the legislation in the past. After the Assembly passed the bill in a special session in early December, Governor Pataki had till the end of the year to sign the bill. His three terms (twelve years) as Governor was ending on December 31st.

With the support of several key insiders in his administration as well as a filicide of media coverage in which support for Timothy's Law became one of the last major issues of Governor Pataki's tenure as Governor, he signed the bill into law on December 22nd of 2006.

The impact of this law was several fold. First, the legislative changes, ended the practice of a sliding scale of payments for mental health services, it dramatically increased access to mental health coverage, it created greater public awareness about the toll of not having coverage for mental health services and most importantly it helped to remedy a wrong that impacted hundreds of thousands of New Yorkers. It also helped to spur continued discussion and support for the important federal legislation that was soon to come.

In many ways the most important lesson learned is the importance of coalition building and grass roots support. If we did not have the leadership of Tom O'Clair and his courage in sharing the story along with tireless partners who were willing to do virtually anything to make this law happen, we would never likely have the law in place. Our Timothy's Law group met weekly then daily and in the last few weeks, we met hourly. There were egos (thankfully we had a leader in Tom O'Clair who had no ego), factions, mis-information, saber rattling, but like a family, which we had become, we put aside all our differences to put our collective energy, passion and wisdom together to fight for full parity.

We all recognized that Timothy's Law was a victory for all New Yorkers, but the enduring legacy is the continued fight among advocates to end discrimination of mental health services in any form.



MHA in Indian River County: Mental Health Walk-In Center

Since 1978, the Mental Health Association in Indian River County (MHA) located in Florida, has been active in mental health education and advocacy. Through the years, the MHA has partnered with health care agencies in the community, local government entities, community leaders, and funders to advocate for people with mental illness, educate the community about mental illness, and provide services focused on teaching people how to handle life's challenges, build strong relationships, and recover from setbacks.

In 2005 the MHA and the Indian River County Mental Health Collaborative worked together to design a Mental Health Walk-In Center to fill the gap in mental health services for children and adults in need of immediate access to mental health care, regardless of ability to pay. Opened in 2007, the MHA Walk-In Center was designed to provide immediate access to emotional and behavioral healthcare in the county, reduce emergency department visits for mental health services at Indian River Medical Center, and reduce psychiatric hospitalizations at the Behavioral Health Center. The Center collaborates with both facilities to ensure an efficient and effective continuum of emotional and behavioral health care.

The Walk-In Center offers emotional and behavioral health care provided by licensed professionals, to the indigent, uninsured and underinsured, and others who may have difficulty navigating the mental health system. Services include screening to establish needs, information and referral, risk assessment, crisis intervention, individual psychotherapy, case management, psychiatric evaluation, and psychiatric medication management and educational classes for anger management, anxiety, depression and domestic violence. The Center is staffed with licensed mental health professionals and licensed psychiatrists. Approximately 9,000 mental health services were provided by mental health therapists and 800 psychiatry services were provided by psychiatrists at the Walk-In Center in 2014.

Providing mental health treatment at a Walk-In Center in an outpatient setting substantially reduces costs within a community. The average cost per client who receives both therapy and psychiatry services at the Walk-In Center is $2,000.00 per course of care, as opposed to the average cost per client of a psychiatric hospitalization of $1500.00 per day or $10,500.00 per week.

Demonstrating efficiency and effectiveness is essential to any program providing behavioral and emotional health services. A recent random sample of clinical records at the Walk-In Center indicated that the average wait time for a client without an appointment to receive a mental health screening was just 12 minutes. Clients receiving psychotherapy and/or psychiatric services at the center on an appointment basis moved from the severe range to the moderate range on self-report measures of depression and anxiety during their treatment. 82% of Walk-In Center clients who completed a voluntary client satisfaction survey strongly agreed or agreed with the statement, "If it weren't for the Walk-In Center I wouldn't have gotten any help." 100% of clients strongly agreed or agreed that with the statement, "My treatment needs are being met at the Walk-In Center."

There is no other program in the area providing immediate access to mental health care in a manner similar to the MHA Walk-In Center. The MHA has made and will continue to make a positive change in Indian River County by creating lasting partnerships with organizations to further our mission and vision. We are committed to providing those who suffer with mental illness, the opportunity for an improved quality of life through access to a continuum of care, reduced stigma of mental health issues and increased community awareness. "It's Okay to Get Help!"

MHA in Delaware: Peer-Led Support Groups

The Mental Health Association in Delaware has been providing peer-led support groups throughout the state for over twenty years. While the efficacy of such groups has been established through research (Pfieffer et al, 2010), the state did not have any groups specifically for depression until MHA began to establish them. We currently have peer led groups for depression, anxiety, and Survivors of Suicide (SOS) throughout Delaware, and professionally led Survivors of Accidents and Murders (SAM) groups. Support groups are a place where people are able to share experiences and compassionately offer encouraging words; a place where people receive help and support from one another.

MHA in Delaware provides these groups because we believe in the effectiveness and the importance of social support systems and meaningful community resources. MHA support groups are offered to assist people as part of their journey towards recovery and total well being. Groups are intended to supplement, not to replace, professional mental health treatment.

All MHA support group meetings are free and confidential. Often, as members begin to share their experiences and feelings, they begin to realize that they are not alone and that they have the power to resolve issues with the support of the group.

A typical support group meeting begins with brief introductions followed by group discussions of related concerns and experiences. Most of MHA's support groups are led by trained, peer volunteers, with the exception of the Survivor's of Accidents and Murders (SAM) Groups, which are facilitated by mental health professionals. Volunteers who have attended the groups and found them helpful will often want to give back by becoming a facilitator.

Each year, MHA evaluates the groups through an anonymous questionnaire that group members fill out. The questionnaire was developed by Cynthia Zubritsky, PhD, a researcher at the University of Pennsylvania. It asks participants to respond regarding their experiences before joining the group, and then after. This allows us to see if the groups are improving the group members' experiences related to their mental health.

Annually, support groups serve over 300 individuals and provide over 3000 total service hours. Last year, 80% of group members reported improvement in their ability to challenge and change negative thinking patters after joining support groups. Over 90% reported improvement in effectively coping with symptoms, and over 95% reported improvement in feeling comfortable to express themselves as needed. The evaluations also showed improvement in adhering to treatment regarding both attending therapy regularly and following instructions on medication, ultimately improving the efficacy of treatment and quality of life for group members.

Because groups rely on volunteer facilitators and are advertised through community connections with partial hospital programs, mental health practices, etc., they are very cost effective. Individuals receive the ancillary support through the groups that helps them to maintain their treatment, improving their health outcomes and ultimately reducing the social costs of the disorder.

The anecdotal comments we often receive on the evaluations are also very encouraging. A few examples are as follows: "This group has been extremely beneficial to my overall wellbeing." "Very thankful this group exists. Probably saved my life." "It has been a major part of my continuing recovery."

One group member shared a story, saying, "Through the MHA sponsored support groups, I have found friends, a support system, and doctors. Participating in groups and sharing with members experiencing the same feelings as me helped immensely, provided an emotional anchor, and helped me learn new coping skills."

MHA in Delaware is privileged to be able to continue these groups which are so helpful to many in our state. We plan to sustain them for many more years to come.